

Aetna Medicare Part B Drug Criteria

Herceptin and Trastuzumab Biosimilars

This policy is for Aetna Medicare members. [Find the Aetna Commercial Medical Drug Criteria.](#)

For Aetna Medicare members, National Coverage Determinations (NCDs) and Local Coverage Determinations (LCDs) will be applied to Part B drug requests when applicable. Aetna Medicare Part B Drug Criteria documents will be used in the absence of an NCD and LCD.

POLICY

Products Referenced by this Document

Drugs that are listed in the following table include both brand and generic and all dosage forms and strengths unless otherwise stated. Over-the-counter (OTC) products are not included unless otherwise stated.

Brand Name	Generic Name
Herceptin	trastuzumab
Ogivri	trastuzumab-dkst
Kanjinti	trastuzumab-anns
Trazimera	trastuzumab-qyyp
Herzuma	trastuzumab-pkrb
Ontruzant	trastuzumab-dttb
Hercessi	trastuzumab-strf

Indications

The indications below including FDA-approved indications and compendial uses are considered a covered benefit provided that all the approval criteria are met and the member has no exclusions to the prescribed therapy.

FDA-Approved Indications¹⁻⁷

Adjuvant Breast Cancer

Adjuvant treatment of human epidermal growth factor receptor 2 (HER2)-overexpressing node positive or node negative (estrogen receptor [ER]/progesterone receptor [PR] negative or with one high risk feature) breast cancer

- as part of a treatment regimen consisting of doxorubicin, cyclophosphamide, and either paclitaxel or docetaxel
- as part of a treatment regimen with docetaxel and carboplatin

- as a single agent following multi-modality anthracycline based therapy

Metastatic Breast Cancer

- In combination with paclitaxel for first-line treatment of HER2-overexpressing metastatic breast cancer
- As a single agent for treatment of HER2-overexpressing breast cancer in patients who have received one or more chemotherapy regimens for metastatic disease

Metastatic Gastric Cancer

In combination with cisplatin and capecitabine or 5-fluorouracil, for the treatment of patients with HER2-overexpressing metastatic gastric or gastroesophageal junction adenocarcinoma who have not received prior treatment for metastatic disease

Compendial Uses⁸⁻¹¹

- HER2-positive breast cancer
 - Neoadjuvant therapy
 - Treatment of recurrent, advanced, unresectable, or stage IV (M1) disease
 - Treatment for no response to preoperative systemic therapy
- HER2-negative breast cancer treatment of stage IV (M1) disease
- Intra-cerebrospinal fluid (CSF) treatment of leptomeningeal metastases (malignant meningitis) from HER2-positive breast cancer
- HER2-positive esophageal and esophagogastric junction cancer
- HER2- positive uterine serous carcinoma and carcinosarcoma
- HER2-positive salivary gland tumor
- HER2-amplified/positive and RAS and BRAF wild-type colorectal cancer
- HER2-positive biliary tract cancers
- HER2-positive non-small cell lung cancer
- Prostate cancer

All other indications will be assessed on an individual basis. Submissions for indications other than those listed in this criteria should be accompanied by supporting evidence from Medicare approved compendia.

Documentation

The following documentation must be available, upon request, for all submissions:

- Human epidermal growth factor receptor 2 (HER2) status, where applicable
- RAS mutation status, where applicable
- BRAF mutation status, where applicable

Coverage Criteria

Breast Cancer¹⁻⁹

Authorization of up to 12 months may be granted for neoadjuvant treatment of HER2-positive breast cancer as part of a complete treatment regimen.

Authorization of up to 12 months may be granted for adjuvant treatment of HER2-positive breast cancer.

Authorization of 12 months may be granted for treatment of HER2-positive breast cancer with no response to preoperative systemic therapy, recurrent, advanced, unresectable, or metastatic (including brain metastases) disease.

Authorization of 12 months may be granted for intra-CSF treatment of leptomeningeal metastases (malignant meningitis) from HER2-positive breast cancer.

Authorization of 12 months may be granted for treatment of HER2-negative metastatic breast cancer when used in combination with neratinib and fulvestrant as third-line or later therapy.

Esophageal, Gastric, or Gastroesophageal Junction Cancer¹⁻⁹

Authorization of 12 months may be granted for treatment or palliative therapy of HER2-positive esophageal, gastric, or esophagogastric junction cancer in combination with chemotherapy.

Uterine Serous Carcinoma or Carcinosarcoma⁸

Authorization of 12 months may be granted for treatment of HER2-positive stage III-IV, metastatic, or recurrent uterine serous carcinoma or carcinosarcoma in combination with carboplatin and paclitaxel and continued as a single agent for maintenance therapy.

Salivary Gland Tumor^{8,11}

Authorization of 12 months may be granted for treatment of recurrent, unresectable, or metastatic HER2-positive salivary gland tumors when used as a single agent or in combination with docetaxel or pertuzumab.

Colorectal Cancer^{8,10}

Authorization of 12 months may be granted for treatment of unresectable, inoperable, advanced, or metastatic colorectal cancer, including appendiceal adenocarcinoma and anal adenocarcinoma, when all of the following criteria are met:

- Member has HER2-positive/amplified disease.
- The disease is negative (wild-type) for RAS (KRAS and NRAS) and BRAF mutations.
- The requested medication will be used in combination with tucatinib, pertuzumab, or lapatinib.
- Member has received prior therapy for the disease or is not appropriate for intensive therapy or will receive hepatic artery infusion.

Biliary Tract Cancers⁸

Authorization of 12 months may be granted for subsequent treatment of unresectable, resected gross residual (R2), or metastatic HER2-positive (IHC 3+/ISH +/-NGS amplification) biliary tract cancers (including intrahepatic and extrahepatic cholangiocarcinoma and gallbladder cancer) when used in combination with pertuzumab or tucatinib.

Non-Small Cell Lung Cancer⁹

Authorization of 12 months may be granted for treatment of HER2-positive (IHC 3+, 2+, or 1+) non-small cell lung cancer.

Prostate Cancer⁹

Authorization of 12 months may be granted for treatment of prostate cancer.

Continuation of Therapy

All members (including new members) requesting authorization for continuation of therapy must be currently receiving therapy with the requested medication.

Authorization of 12 months may be granted when all of the following criteria are met:

- The member is currently receiving therapy with the requested medication.
- The requested medication is being used to treat a diagnosis or condition listed in the coverage criteria section.
- For members requesting reauthorization for adjuvant or neoadjuvant treatment of breast cancer, the maximum treatment duration is 12 months.
- The member is receiving benefit from therapy. Benefit is defined as:
 - No evidence of unacceptable toxicity while on the current regimen AND
 - No evidence of disease progression while on the current regimen.

Summary of Evidence

The contents of this policy were created after examining the following resources:

- The prescribing information for Herceptin, Kanjinti, Ogivri, Trazimera, Herzuma, Ontruzant, and Hercessi
- The available compendium
 - National Comprehensive Cancer Network (NCCN) Drugs and Biologics Compendium
 - Micromedex DrugDex
 - American Hospital Formulary Service- Drug Information (AHFS-DI)
 - Lexi-Drugs
 - Clinical Pharmacology
- NCCN Guideline: Breast Cancer
- NCCN Guideline: Biliary Tract Cancers
- NCCN Guideline: Central Nervous System Cancers
- NCCN Guideline: Colon Cancer
- NCCN Guideline: Esophageal and Esophagogastric Junction Cancers
- NCCN Guideline: Gastric Cancer
- NCCN Guideline: Head and Neck Cancers
- NCCN Guideline: Rectal Cancer
- NCCN Guideline: Uterine Neoplasms

After reviewing the information in the above resources, the FDA-approved indications listed in the prescribing information for Herceptin, Kanjinti, Ogivri, Trazimera, Herzuma, Ontruzant, and Hercessi are covered in addition to the following:

- HER2-positive breast cancer:
 - Neoadjuvant therapy
 - Treatment of recurrent, advanced, unresectable, or stage IV (M1) disease
 - Treatment for no response to preoperative systemic therapy
- HER2-negative breast cancer treatment of stage IV (M1) disease
- Intra-cerebrospinal fluid (CSF) treatment of leptomeningeal metastases (malignant meningitis) from HER2-positive breast cancer
- HER2-positive esophageal and esophagogastric junction cancer
- HER2- positive uterine serous carcinoma and carcinosarcoma
- HER2-positive salivary gland tumor
- HER2-amplified/positive and RAS and BRAF wild-type colorectal cancer
- HER2-positive biliary tract cancers
- HER2-positive non-small cell lung cancer
- Prostate cancer

Explanation of Rationale

Support for FDA-approved indications can be found in the manufacturer's prescribing information.

Support for all indications other than non-small cell lung cancer and prostate cancer can be found in the NCCN Drugs and Biologics Compendium. Use of information in the NCCN Drugs and Biologics Compendium for off-label use of drugs and biologicals in an anti-cancer chemotherapeutic regimen is supported by the Medicare Benefit Policy Manual, Chapter 15, section 50.4.5 (Off-Label Use of Drugs and Biologicals in an Anti-Cancer Chemotherapeutic Regimen).

Support for non-small cell lung cancer and prostate cancer can be found in the Micromedex DrugDex database. Use of information in the DrugDex database for off-label use of drugs and biologicals in an anti-cancer chemotherapeutic regimen is supported by the Medicare Benefit Policy Manual, Chapter 15, section 50.4.5 (Off-Label Use of Drugs and Biologicals in an Anti-Cancer Chemotherapeutic Regimen).

References

1. Herceptin [package insert]. South San Francisco, CA: Genentech, Inc.; June 2024.
2. Kanjinti [package insert]. Thousand Oaks, CA: Amgen Inc.; June 2025.
3. Ogivri [package insert]. Cambridge, MA: Biocon Biologics Inc.; November 2024.
4. Trazimera [package insert]. Cork, Ireland: Pfizer Ireland Pharmaceuticals; November 2020.
5. Herzuma [package insert]. Incheon, Republic of Korea: Celltrion, Inc.; December 2024.
6. Ontruzant [package insert]. Jersey City, NJ: Organon LLC; February 2025.
7. Hercessi [package insert]. Raleigh, NC: Accord BioPharma Inc.; September 2024.
8. The NCCN Drugs & Biologics Compendium® © 2025 National Comprehensive Cancer Network, Inc. <https://www.nccn.org>. Accessed September 5, 2025.

9. Micromedex® (electronic version). Merative, Ann Arbor, Michigan, USA. Available at: <https://www.micromedexsolutions.com>. Accessed September 5, 2025.
10. National Comprehensive Cancer Network. NCCN Clinical Practice Guidelines in Oncology: Anal Carcinoma. Version 4.2025. Available at: https://www.nccn.org/professionals/physician_gls/pdf/anal.pdf. Accessed September 5, 2025.
11. National Comprehensive Cancer Network. NCCN Clinical Practice Guidelines in Oncology: Head and Neck Cancers. Version 5.2025. Available at: https://www.nccn.org/professionals/physician_gls/pdf/head-and-neck.pdf. Accessed September 3, 2025.

DOCUMENT HISTORY

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See Evidence of Coverage for a complete description of plan benefits, exclusions, limitations and conditions of coverage. Plan features and availability may vary by service area.